

Virginia Rollout Plan + KPIs

One dominant partner per city. Start with two cities — prove the model — then scale off the case study.

The Targeting Principle

There is one dominant chiropractor in every ~150,000-population town — the longest-tenured, college / sports / personal-injury DC everyone refers to. **We partner with that ONE practice per city, not a cluster.**

"There's always one guy — he's the college guy, the sports guy, the personal-injury guy, been there the longest, everybody refers to him." — Dr. Wagner

Target Cities (tiered)

City	Tier	Role in the rollout
Waynesboro · Staunton · Harrisonburg · Lynchburg	1 · Start here	Smaller college towns — fewer strong clinics, so the one dominant practice is easier to find and sign. (Charlottesville is excluded — the principal can't stay anonymous in his home market.)
Richmond	1 · Test #2	Top candidate for the second test city — largest VA chiropractor population, best paid-ad efficiency.
Virginia Beach	1 · Test #2 (alt)	Alternate second test city — dense market.
Fairfax / NoVA	3	Dense but higher ad costs; sequence after the model is proven.
Fredericksburg · Harrisonburg · Lynchburg · Roanoke	3	Mid-markets; one partner each as we scale.
Williamsburg · Waynesboro · Orange	4	Long tail — reach primarily via the owned email list + referrals.

Sequencing (lean & conservative)

- **Month 1 — Lean test:** concentrate the full ad budget + the owned-list email burst on **the focus cluster — Waynesboro, Staunton, Harrisonburg, Lynchburg (+ Richmond candidate)**. Goal: a clean cost-per-application and momentum toward **one signed partner per city**.
- **Months 2-3 — Prove:** drive both test cities toward the 10-20 application floor and sign 1-2 partners. Build the case study.
- **Months 4+ — Scale:** use the signed case study to justify scaling spend city-by-city across the remaining Virginia cities.

Reality check: a \$5,000/mo ad budget cannot produce 10-20 applications across all of Virginia at once — that would need roughly \$45-90K/mo against an audience too thin to absorb it. **Concentrate on one city first.** (See the Engagement & Budget.)

Goals

1 dominant partner signed per city (the goal)	10-20 qualified applications per city to filter to one	~\$450 target cost per qualified application (planning)	<1 hr speed-to-lead on every application
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Funnel KPIs (per city)

Funnel stage	Target
Landing/ad click → intake started	~10% of clicks
Intake started → application completed	25-35%
Completed → qualified (meets the gate below)	40-50%
Qualified → discovery call booked	≥ 60%
Discovery → lease partnership signed	1 strong partner / city
Cost per qualified application	\$350-600 (plan ~\$450)

The "Good Partner" Gate (qualify criteria)

- **45+** and growth-minded; **25+ new patients/mo** (2-year baseline, not a spike)
- **Dominant** local reputation (sports / PI / college DC, longest-tenured) with strong retention + care plans
- **Spare space** for a medical partnership
- Still treats Medicare / has cash-service solutions (altruistic *and* can sell)

- **Exit-minded** — thinks of the practice as a business, feeling the retirement / college squeeze

Month-1 Success Criteria (reset expectations)

Not "10-20 apps in all 11 cities." **Yes:** drive the **focus cities** (Waynesboro, Staunton, Harrisonburg, Lynchburg) toward the 10-20 application floor, establish a real cost-per-application, and **sign the first partner** — the proof that scales the rest of the state.

PROMEDVA · Confidential. Targeting reflects Dr. Wagner's stated criteria from the 2026-06-06 strategy call. Figures are planning estimates, not guarantees. Ad targeting executed within platform policy (no discriminatory targeting in ad copy).